Fax cover sheet

Harbor View Medical Center (fictional)

Case Management Department

FAX TRANSMITTAL

To: Illumination Health + Home - Recuperative Care Intake
Fax: 555-0199
From: Janet Okafor, RN CM
Phone: 555-0110
Date: 03/05/2026 09:20
Pages (including cover): 11
Re: Recuperative care referral
Urgent [X] For Review [X] Please Reply [X]

Message

Referral for recuperative care for 46F, LLE cellulitis, improving on PO abx. Anticipated d/c 03/05 PM. Home health (Coastline Home Health) SOC 03/06 for wound checks.
Please call CM with questions.

CONFIDENTIALITY NOTICE

This facsimile contains confidential health information. If received in error, notify sender and destroy.

-- FICTIONAL DEMO RECORD - not a real patient --

IHH Recuperative Care Referral Form

ILLUMINATION HEALTH + HOME

Recuperative Care Referral Form (RC0725-SH)

Referring Party

Hospital: Harbor View Medical Center (fictional)
Department/Floor: 4 West Med-Surg
Social Worker/Case Manager/Nurse: Janet Okafor, RN CM
Phone: 555-0110
Email: jokafor@harborview.example
Nursing Station: 555-0111
Authorized By: Dr. P. Lindqvist

Patient Information

Name: Rosa Doe
MRN: HV-0048213
Birth Date: 08/14/1979
Gender: F
Ethnicity: Hispanic/Latina
SSN (last 4): 0000
Insurance: Orange Coast Health Plan (fictional) Medi-Cal
Preferred Language: English
Housing Situation:
Currently Unhoused [X]
4+ episodes of homelessness in the past 3 years [ ]

Hospital Admission Details

Chief Complaint/Admitting Diagnosis: Cellulitis L lower leg
Admit Date: 03/02/2026
Any Wounds: Yes [ ] No [X]
Number/Location/Size/Stage:
Independent with Wound Care: Yes [X] No [ ]
Home Health Ordered: Yes [X] No [ ]
PPD/TB Test or Chest X-Ray: Date: 03/02/2026 Result: CXR neg - no acute process

History of Health & Living Situation

O2: Yes [ ] No [X]
Self Administer All Meds: Yes [X] No [ ]
Continent of Bowel & Bladder: Yes [X] No [ ]
Colostomy/Ileostomy: Yes [ ] No [X]
Foley Catheter: Yes [ ] No [X]
Diabetes: Yes [ ] No [X]
Insulin: Yes [ ] No [X]
Diabetes Oral Meds: Yes [ ] No [X]
Active Infection: Yes [X] No [ ] cellulitis on PO abx
Ambulatory: Yes [X] No [ ]
Assistive Device: Yes [ ] No [X]
Hospice: Yes [ ] No [X]
Car: Yes [ ] No [X]
Service Animal: Yes [ ] No [X]

Behavioral and Mental Health Status

Any limitations or challenges: No [X] Yes [ ]

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CalAIM Community Supports authorization request

CalAIM COMMUNITY SUPPORTS - AUTHORIZATION REQUEST

Service: Recuperative Care (Medical Respite)
Health Plan: Orange Coast Health Plan (fictional) - Medi-Cal

Member

Member Name: Rosa Doe
DOB: 08/14/1979
Member ID: OCH-77120456

Referring Provider

Facility: Harbor View Medical Center (fictional)

Clinical

Primary Diagnosis: Cellulitis of left lower limb
ICD-10: L03.116
Member is experiencing homelessness [X]
Member at risk of hospitalization/re-hospitalization [X]
Requested Start Date: 03/05/2026
Requested Days: 14
Clinical summary: Cellulitis LLE improving on PO antibiotics. Independent ADLs.

Plan Use Only

Status: Pending review

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Patient face sheet

Harbor View Medical Center (fictional) - PATIENT FACE SHEET

Patient: Rosa Doe
MRN: HV-0048213
Account #: 99048213
DOB: 08/14/1979
Sex: Female
Preferred Language: Spanish / English (bilingual)
Ethnicity: Hispanic or Latino
Marital Status: Single
Address: NO FIXED ADDRESS (homeless)
Mailing: 1234 Example Ave, Santa Ana, CA 92700 (shelter mail drop)
Phone: 555-0100 (message)

Encounter

Admit Date/Time: 03/02/2026 16:48
Admit Type: Inpatient
Unit/Bed: 4W-412B
Attending: Priya Lindqvist, MD

Insurance

Primary: Orange Coast Health Plan (fictional) - Medi-Cal
Subscriber: Self

Emergency Contact

None listed

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History and Physical - page 1

HISTORY AND PHYSICAL

Date of Service: 03/02/2026
Attending: Priya Lindqvist, MD (Hospital Medicine)

Chief Complaint

"My left leg is red and swollen" x 4 days

HPI

46 y/o F experiencing homelessness (sleeps in vehicle-less encampment near riverbed) with no significant PMH presents with 4 days of progressive erythema, warmth and pain of the L anterior shin after a scrape on a fence. Subjective fevers/chills x 2 days. Denies IVDU. No prior MRSA infection known. In ED: T 101.2F, HR 108, WBC 14.2. Received vancomycin 1.5 g IV x1 and cefazolin 2 g IV.

PMH

- Seasonal allergies
- No DM, no HTN

PSH

- C-section 2008

Social History

Homeless x 18 months. Tobacco 1/2 ppd x 20 yrs. EtOH: social, last drink 1 wk ago. Denies drug use.

Allergies

NKDA

Home Medications

None

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History and Physical - page 2

Physical Exam

VS: T 100.8F, HR 102, BP 128/76, RR 18, SpO2 98% RA
Gen: NAD, AOx4
CV: RRR, no m/r/g
Lungs: CTAB
Skin: L anterior shin erythema approx 14 x 9 cm, marked with pen, warm, tender. Central superficial open area 2.0 x 1.5 cm, minimal serous drainage, no fluctuance. No crepitus.
Ext: 1+ edema LLE, DP pulses 2+ bilat
Neuro: grossly intact

Assessment / Plan

46F with non-purulent cellulitis L lower leg with small superficial ulceration, mild SIRS on arrival.
1. Cellulitis LLE - IV cefazolin 2 g q8h; LE venous duplex to r/o DVT; elevate; wound care consult; blood cx x2
2. MRSA nares PCR, COVID per protocol
3. Tobacco use - nicotine patch 14 mg offered
4. Dispo - CM consult; homeless, anticipate recuperative care referral
DVT ppx: enoxaparin 40 mg SC daily
Code status: Full
Electronically signed: Priya Lindqvist, MD 03/02/2026 21:14

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Wound care nurse note

WOUND CARE CONSULT NOTE

Date: 03/04/2026 11:05
Wound #1: L anterior shin, superficial open area within cellulitis
Size: 1.8 cm L x 1.2 cm W x 0.1 cm D (improved from 2.0 x 1.5)
Wound bed: 90% pink granulation, 10% yellow slough
Drainage: scant serous. Odor: none
Periwound: erythema receding, now inside marked borders

Treatment

- Cleanse with NS, apply hydrogel, cover with foam dressing
- Change dressing every other day and PRN

Education

Pt return-demonstrated dressing change independently x2. Verbalized s/s of worsening infection.

Recommendation

Home health RN for wound check 2x/week x 2 weeks.
Signed: L. Ferreira, RN CWOCN

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Hospitalist progress note and CM note

HOSPITALIST PROGRESS NOTE - HD#4

Date: 03/05/2026 07:40
S: Pain 2/10, ambulating halls independently. No fevers x 48h.
O: T 98.4F HR 78 BP 122/74 RR 16 SpO2 99% RA
Erythema markedly receding. Duplex 03/03 neg for DVT.
Blood cx 03/02 x2: no growth at 72h.
A/P: Cellulitis LLE improving.
- Transition to PO cephalexin 500 mg QID + TMP-SMX DS 1 tab BID to complete 7 days total
- Medically stable for d/c to recuperative care today

CASE MANAGEMENT NOTE

Date: 03/05/2026 08:55
Pt AOx4, independent with ADLs and ambulation (no device). Pt agreeable to recuperative care.
Home health referral accepted by Coastline Home Health (fictional), 555-0131; RN SOC visit scheduled 03/06/2026 for wound check, then 2x/week.
Abx script to be filled at bedside pharmacy (30-day supply policy n/a; 7-day course).
Signed: Janet Okafor, RN CM

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Vital signs flowsheet

VITAL SIGNS - LAST 5 SETS

Date/Time | Temp (F) | HR | BP | RR | SpO2 | O2
03/03 20:00 | 99.1 | 88 | 126/78 | 18 | 98% | RA
03/04 08:00 | 98.6 | 84 | 124/76 | 16 | 99% | RA
03/04 16:00 | 98.7 | 80 | 120/72 | 16 | 98% | RA
03/05 00:00 | 98.2 | 76 | 118/70 | 16 | 99% | RA
03/05 07:40 | 98.4 | 78 | 122/74 | 16 | 99% | RA
Pain 2/10 L shin. Ambulating independently.

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Laboratory results

LABORATORY RESULTS

Collected | Test | Result | Ref
03/02 14:30 | WBC | 14.2 H | 4.0-11.0 K/uL
03/04 05:00 | WBC | 9.1 | 4.0-11.0 K/uL
03/04 05:00 | Hgb | 12.8 | 12.0-16.0 g/dL
03/04 05:00 | Plt | 288 | 150-400 K/uL
03/04 05:00 | Creatinine | 0.7 | 0.5-1.1 mg/dL
03/04 05:00 | Glucose | 96 | 70-99 mg/dL
03/02 14:30 | Lactate | 1.4 | 0.5-2.0 mmol/L

MICROBIOLOGY

03/02 Blood culture x2: No growth at 72 hours (final pending 5 days)
03/03 06:15 MRSA nares PCR: NEGATIVE
03/05 06:10 SARS-CoV-2 rapid antigen: NEGATIVE

IMAGING

03/02 CXR PA/Lat: No acute cardiopulmonary process
03/03 LLE venous duplex: No DVT

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Discharge medication list

DISCHARGE MEDICATION RECONCILIATION

Patient: Rosa Doe MRN: HV-0048213

New Medications

- Cephalexin 500 mg PO four times daily x 4 more days (last dose 03/08)
- Sulfamethoxazole-trimethoprim 800-160 mg (DS) 1 tab PO twice daily x 4 more days
- Acetaminophen 650 mg PO every 6 hours PRN pain (max 3 g/day)
- Nicotine patch 14 mg/24h, apply daily

Wound Supplies

- Hydrogel, foam dressings 4x4 (10 count), NS

Stopped

- Cefazolin IV (inpatient only)
- Enoxaparin (inpatient only)

Patient able to self-administer all medications: Yes

Reviewed with patient by: M. Tran, PharmD 03/05/2026

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